24

24 Maternity Care

Pregnancy related care for Medicaid eligible women provided through the Maternity Care Program (MCP) is provided pursuant to the Alabama State Plan as approved by the U.S. Department of Health and Human Services Centers for Medicare and Medicaid Services (CMS) and the approved 1915(b) Waiver. The purpose of the program is to provide a comprehensive, coordinated system of obstetrical care to pregnant recipients.

NOTE:

A confirmatory positive pregnancy test is not required for maternity Medicaid eligibility.

24.1 Maternity Care Providers (MCPs)

24.1.1 Enrollment

The Alabama Medicaid Agency fiscal agent enrolls physicians and issues provider contracts to applicants who meet the licensure and/or certification requirements of the state of Alabama, the Code of Federal Regulations, the *Alabama Medicaid Agency Administrative Code*, and the *Alabama Medicaid Provider Manual*. For the purpose of enrollment, a physician is defined as: a physician who is fully licensed and possesses a current license to practice medicine.

Refer to Chapter 2, Becoming a Medicaid Provider, for general enrollment instructions and information. Failure to provide accurate and truthful information or intentional misrepresentation might result in action ranging from denial of application to permanent exclusion.

Federal requirements mandate providers re-validate periodically with the Alabama Medicaid program. Providers will be notified when they are scheduled to re-validate. Failure to re-validate and provide appropriate documentation to complete enrollment will result in an end-date being placed on the provider file. Once a provider file has been closed for failure to timely re-validate, providers will have to submit a new application for enrollment.

Providers who contract with Alabama Medicaid as a OB/GYN are added to the Medicaid system with the National Provider Identifiers provided at the time application is made. Appropriate provider specialty codes are assigned to enable the provider to submit requests and receive reimbursements for Maternity Care related claims.

NOTE:

The 10-digit NPI is required when filing a claim.

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Physicians are assigned a provider type of 31 (Physician). OB/GYNs are assigned a provider specialty of 328.

ACHN MCP Agreement: All MCPs, including Maternal Fetal Medicine (MFM) and telemedicine providers, must sign one additional agreement beyond their Medicaid Enrollment. The MCP must sign an agreement with an ACHN in order to receive reimbursement of maternity services and bonus payments. On a monthly basis, ACHNs will notify the Agency of all executed ACHN MCP agreements. MCPs and ACHNs must ensure that the Medicaid Group Billing ID, NPI, Medicaid ID, and name listed on the MCP agreement is correct and consistent with what the Agency has on the provider's Medicaid file. The provider's file must also be in an active status with the Medicaid Agency. All information submitted must be based on the group level unless the MCP is set up as an individual practice. If the information is not correct or consistent, the agreement will not be added to the provider's Medicaid file. In the absence of this agreement, MCPs will not be eligible for reimbursement for maternity services and will not receive bonus payments for performing first trimester and post-partum visits. Contact the ACHN you intend to participate with to inquire about submission deadlines for the MCP Agreement.

The enrollment effective date for the ACHN MCP Agreement will the first day of the following month, if the agreement is received and contains no errors prior to the 15th of the month. For agreements received on or after the 15th of the month, the effective date of the enrollment will be the month following the next month.

The MCP group must work with the ACHN to ensure timely submission of the MCP agreement. Contact the ACHN you intend to participate with to inquire about submission deadlines for the MCP agreement.

To obtain the MCP Group Agreement with the ACHN, email the ACHN Regional contact. Visit www.Medicaid.Alabama.gov and select the ***ACHN Providers*** tab, under Program Documents for the ACHN Contact Information for Providers.

NOTE:

To receive reimbursement for maternity related services, all Maternal Fetal Medicine (MFM) and telemedicine providers must sign one additional agreement beyond their Medicaid Enrollment. The MCP agreement must be signed with one of the ACHNs. (Refer to Chapter 112, Telemedicine Services, for general benefit information and limitations.)

24.2 Telemedicine Providers (for MFMs only)

Telemedicine providers who render maternity related services are required to sign a MCP agreement with one of the ACHNs to receive reimbursement from Medicaid. Refer to Chapter 112 of the Provider Billing Manual to determine further requirements and procedure codes for telemedicine services.

Beginning July 8, 2020, Medicaid will allow physicians enrolled with the specialties OB/GYN (specialty type 328) and telemedicine (specialty type

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931) to be reimbursed for maternity services with a referral from either an ACHN or the referring MCP. The NPI of the ACHN, the MCP, or the referring MCP's group must be on the claim for reimbursement. Refer to MCP Selection Referral Number section for more detailed billing information.

Telemedicine providers who render maternity-related services are required to sign a Non-Delivering Telemedicine MCP Participation agreement with at least one of the ACHNs in order to receive reimbursement from Medicaid. Refer to Chapter 112 of the Provider Billing Manual to determine further requirements and procedure codes allowed for telemedicine services.

NOTE:
Refer to Chapter 112, Telemedicine Services, for general benefit information and limitations.

24.3 ACHN Contact Information (for MCPs)

Visit www.Medicaid.Alabama.gov and select the ***ACHN Providers*** tab, under Program Documents for the ACHN Contact Information for Providers.

24.4 MCPs Benefits and Limitations

This section describes program-specific benefits and limitations. Refer to Chapter 3, Verifying Recipient Eligibility, for general benefit information and limitations. Refer to Chapter 7, Understanding Your Rights and Responsibilities as a Provider, for general criteria on Medical Necessity/ Medically Necessary Care.

NOTE:

Medicaid SOBRA (Pregnant Women) recipients, who were once eligible for pregnancy-related services ONLY may receive full Medicaid benefits throughout pregnancy and post-partum, whether the services were pregnancy related or not. A Primary Care Physician (PCP) referral is NOT required to receive non-pregnancy related services.
Claims that are pregnancy related will require a pregnancy related diagnosis code or a postpartum diagnosis code. Co-pays may be applied for services that are non-pregnancy related. A recipient's age, health care requirements, and place of residence may further define his or her eligibility for Medicaid covered services. For this reason, it is very important that the providers verify recipient eligibility and ensure they understand all aspects of the eligibility response.

Eligibility responses have been changed to reflect the correct coverage for these women.

Refer to Chapter 4, Obtaining Prior Authorization (PA) for PA requirements.

Recipients are notified at the time of Medicaid application of the requirement to participate in the ACHN Program.

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24.5 Recipient County Moves and Changes

Maternity recipients must receive a MCP Selection Referral number from their assigned ACHN for maternity related services. In some instances, the maternity recipient may have a different county listed on their eligibility verification. In these situations, the maternity recipient must contact Medicaid to update their address and county. The recipient can update their address and county by calling the recipient call center, complete an update form (Form 295), contact their local caseworker, or use their My Medicaid online account. After the update has been requested, the address and county will update within 24 business hours. However, the ACHN assignment will not change until the following month (if completed on or before the 27th of the month). Until the ACHN assignment is updated, the MCP must work with the ACHN that the recipient is currently assigned to for referrals.

24.6 Presumptive Eligibility

24.6.1 Hospital Presumptive Eligibility

Hospital Presumptive Eligibility (HPE) is temporary Medicaid coverage for up to 60 days. Coverage begins the first day of the month and ends the last day of the following month if the person has not applied for regular Medicaid within the 60 days. The HPE application is approved by the HPE Determiner affiliated with a Qualified Hospital that has elected to participate. If a recipient is approved as pregnancy only, services are limited to ambulatory prenatal and pregnancy-related care only (inpatient expenses are not covered). To access the Patient HPE Application and the HPE Provider Agreement use the following link:
http://medicaid.alabama.gov/content/9.0_Resources/9.4_Forms_Library/9.4.7_Hospital_Forms.aspx

24.6.2 Presumptive Eligibility for Pregnancy (PEP)

The Alabama Legislature passed Senate Bill 102 (Act 2025-204) during the 2025 legislative session establishing Presumptive Eligibility for Pregnancy (PEP) with implementation on October 1, 2025. PEP allow pregnant women early access to ambulatory prenatal care while their application for Medicaid is under review. The major provisions of the law are listed below.

PEP Eligibility Period

The PEP eligibility period begins on the date a determination is made by a qualified provider and ends at or on the earliest of the following:

- The date which the Agency makes a determination of eligibility for coverage based on an application submitted by the woman
- If the woman fails to file an application with the agency for coverage, the last day of the month following the month in which the qualified provider made the determination of presumptive eligibility
- No later than the 60th day after the qualified provider made the determination of presumptive eligibility

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PEP Benefit Plan Covers Ambulatory Prenatal Services

- Professional services (e.g. provider office visits, radiology interpretation)
- Outpatient services (e.g. laboratory studies, radiology studies)
- Pharmacy services

PEP Benefit Plan Does Not Cover

- Inpatient services
- Dental services

Providers Eligible to Become Determiners for PEP

- Hospitals
- County Health Departments
- Federally Qualified Health Centers (FQHCs)
- Rural Health Clinics (RHCs)
- Physicians
- Certified Nurse Midwives (CNMs)
- Certified Registered Nurse Practitioners (CRNPs)
- Physician Assistants (PAs)

Eligibility Criteria for PEP

- Must be pregnant
- Must meet income guidelines
  - Income must not exceed the modified adjusted gross income (MAGI) limit for the eligibility for pregnant women which is in effect under the state Medicaid plan
    - Income limits are updated annually and can be found on the Alabama Medicaid website at the following link: Medicaid Eligibility
- Self-attestation can be accepted for both pregnancy and income verification

Becoming a Qualified Provider/Determiner

- Must complete training provided by Alabama Medicaid
- Must sign an agreement with Alabama Medicaid

Qualified Provider Requirements

- The qualified provider must notify the Agency within five business days of the determination
- The qualified provider should inform the patient that she must apply for Medicaid no later than the last day of the following month
  - Online application for Medicaid: Insure Alabama - Apply for ALL Kids and Medicaid coverage

PEP and Alabama Coordinated Health Network

- Women receiving PEP coverage will not be assigned to an Alabama Coordinated Health Network (ACHN)
- Women receiving PEP coverage will not require an ACHN referral prior to seeing a maternity care provider for prenatal care

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NOTE:

PEP Resources can be found here: Alabama Medicaid Maternity Website

Questions about PEP can be directed to:
pep@medicaid.alabama.gov

24.7 Covered Services

All services provided by the MCP will be billed fee for service.

24.7.1 Antepartum Care

Antepartum care includes the following usual prenatal services:

- Initial visit at the time pregnancy is diagnosed
- Initial and subsequent histories
- Maternity counseling
- Risk assessments
- Physical exams
- Recording of weight
- Blood pressure recordings
- Fetal heart tones
- Lab work appropriate to the level of care including hematocrit and chemical urinalysis

24.7.2 Delivery

Delivery includes vaginal delivery, with or without episiotomy, with or without forceps or cesarean section delivery. More than one fee **may not** be billed for a multiple birth delivery. Delivery includes, but is not limited to, professional services, such as physician's services and anesthesiology. Any non-routine newborn care must be billed under the baby's Medicaid number. Please refer to Chapter 28 for charges that are billable fee-for-service by physicians.

24.7.3 Hospitalization

Hospitalization includes delivery as well as any pregnancy-related hospitalizations that occur in the antepartum period or postpartum period. Hospitalization includes all charges that are normally submitted on the uniform billing claim form (UB-04), which includes but is not limited to the following:

- Labor
- Delivery or operating room
- Room and board including well baby nursery days
- Drugs, supplies, and lab/radiology services obtained during hospitalization

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NOTE:
Physician sterilization charges may be billed fee-for-service.

All outpatient hospital services associated with a pregnancy related condition are to be billed as fee for service by the Provider of service utilizing the most appropriate CPT code. **A pregnancy diagnosis code, primary or secondary, must be used when billing maternity care services.**

24.7.4 Postpartum Care

Postpartum care includes office visits, home visits, and in-hospital visits following delivery for routine care through the end of the month of the postpartum period. The postpartum exam should be accomplished 7 to 84 days after delivery.

It is recommended that providers follow current guidance from the American College of Obstetrics and Gynecology (ACOG) for postpartum care.

ACOG recommends:

- All women should ideally have contact with a maternal care provider within the first 3 weeks postpartum. This initial assessment should be followed up with ongoing care as needed, concluding with a comprehensive postpartum visit no later than 12 weeks after birth.
- The timing of the comprehensive postpartum visit should be individualized and woman centered.
- The comprehensive postpartum visit should include a full assessment of physical, social, and psychological well-being.
- Women with pregnancies complicated by preterm birth, gestational diabetes, or hypertensive disorders of pregnancy should be counseled that these disorders are associated with a higher lifetime risk of maternal cardiometabolic disease.
- Women with chronic medical conditions, such as hypertensive disorders, obesity, diabetes, thyroid disorders, renal disease, mood disorders, and substance use disorders, should be counseled regarding the importance of timely follow-up with their obstetrician–gynecologists or primary care providers for ongoing coordination of care.
- For a woman who has experienced a miscarriage, stillbirth, or neonatal death, it is essential to ensure follow-up with an obstetrician–gynecologist or other obstetric care provider.

24.7.5 Dental Services

Effective October 1, 2022, Alabama Medicaid covers dental services for pregnant adult recipients (over 21). Coverage is effective during pregnancy and through the end of the month of 60 days postpartum. For billing instructions, you may refer to Chapter 13 of the Provider Billing Manual.

24.7.6 Obstetrical and Related Services

The following policy refers to maternity care billed as fee-for-service.

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Physician visits for obstetrical care are counted as part of each recipient's benefit limit of 14 physician visits per year under the conditions listed below.

Maternity Care and Delivery
The services normally provided in maternity cases include antepartum care, delivery, and postpartum care. When a physician provides total obstetrical care, the procedure code which shall be filed on the claim form is the code for all-inclusive "global" care. The indicated date of service on "global" claims should be the date of delivery. If a woman is pregnant at the time she becomes eligible for Medicaid benefits, only those services provided during the time she is eligible will be covered.

NOTE:

When a physician provides eight (8) or more prenatal visits, performs the delivery, and provides the postpartum care, the physician must use a "global" obstetrical code in billing.

If a physician submits a "global" fee for maternity care and delivery, the visits covered by these codes are not counted against the recipient's limit of 14 physician office visits a calendar year. For purposes of "global" obstetrical billing, services rendered by members of a group practice are to be considered as services rendered by a single provider.

NOTE:

The date of service on the “global” OB claim must be the date of delivery.

Antepartum care includes all usual prenatal services such as initial office visit at which time pregnancy is diagnosed, initial and subsequent histories, physical examinations, blood pressure recordings, fetal heart tones, maternity counseling, etc.; therefore, additional claims for routine services should not be filed. Antepartum care also includes routine lab work (e.g., hemoglobin, hematocrit, chemical urinalysis, etc.); therefore, additional claims for routine lab work should not be filed.

To justify billing for global antepartum care services, physicians must utilize the CPT-4 antepartum care global codes (either 4-6 visits or 7 or more visits), as appropriate. Claims for antepartum care filed in this manner do not count against the recipient's limit of 14 office visits per year.

NOTE:

Physicians who provide less than four (4) visits for antepartum care must utilize CPT-4 codes under office medical services when billing for these services. These office visit codes will be counted against the recipient’s limit of 14 physician visits a calendar year.

Billing for antepartum care services in addition to "global" care is not permissible. However, in cases of pregnancy complicated by toxemia, cardiac problems, diabetes, neurological problems or other conditions requiring additional or unusual services or hospitalization, claims for

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additional services may be filed. If the physician bills fragmented services in any case other than high-risk or complicated pregnancy and then bills a "global" code, the fragmented codes shall be recouped. Claims for such services involved in complicated or high risk pregnancies may be filed utilizing CPT codes for Office Medical Services. Claims for services involving complicated or high-risk pregnancies must indicate a diagnosis other than normal pregnancy and must be for services provided outside of scheduled antepartum visits. These claims for services shall be applied against the recipient's limit of 14 physician office visits a calendar year.

NOTE:

Claims submitted by teaching facilities and board certified Perinatologist for services provided for high risk pregnancies must be billed with a TG modifier. Provider Specialty Type 922 is limited to bill three (3) office visits without the TG modifier.

Delivery and Postpartum Care

Delivery shall include vaginal delivery (with or without episiotomy) or cesarean section delivery and all in-hospital postpartum care. More than one delivery fee may not be billed for a multiple birth (twins, triplets, etc.) delivery, regardless of delivery method(s). Delivery fees include all professional services related to the hospitalization and delivery which are provided by the physician; therefore, additional claims for physician's services in the hospital such as hospital admission, may not be filed in addition to a claim for delivery or a claim for "global" care.

EXCEPTION: When a physician's first and only encounter with the recipient is for delivery ("walk-in" patient) he may bill for a hospital admission (history and physical) in addition to delivery charges.

Postpartum care includes office visits following vaginal or cesarean section delivery for routine postpartum care within 62 days post-delivery. Routine postpartum care includes a physical exam as well as family planning services to include prescribing and/or administering contraception. Additional claims for routine visits during this time should not be filed.

Delivery Only

If the physician performs the delivery only, he must utilize the appropriate CPT-4 delivery only code (vaginal delivery only or C-section delivery only). More than one delivery fee may not be billed for a multiple birth (twins, triplets, etc.) delivery, regardless of the delivery method(s). Delivery fees include all professional services related to the hospitalization and delivery which are provided by the physician; therefore, additional claims for physician's services in the hospital such as hospital admission, may not be filed in addition to a claim for delivery only.

EXCEPTION: When a physician's first and only encounter with the recipient is for delivery ("walk-in" patient) he may bill for a hospital admission (history and physical) in addition to delivery charges.

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Ultrasounds

Generally, ultrasounds are conducted to detect gestational age, multiple pregnancies, and major malformations, detect fetal growth disorders (intrauterine growth retardation, macrosomia) and anomalies. All ultrasounds must be medically necessary with medical diagnosis documented supporting the benefit of the ultrasound procedure.

Some medical conditions that are medically necessary are:

- Gestational diabetes with complications (Type 1 diabetes, vascular disease, hypertension, elevated alpha-fetoprotein values, poor patient compliance)
- Failure to gain weight, evaluation of fetal growth
- Pregnancy-induced hypertension
- Vaginal bleeding of undetermined etiology
- Coexisting adnexal mass
- Abnormal amniotic fluid volume (polyhydramnios, oligohydramnios)
- Pregnant trauma patient
- Congenital diaphragmatic hernia (CDH)
- Monitoring for special tests such as fetoscopy, amniocentesis, or cervical cerclage placement
- Assist in operations performed on the fetus in the uterus
- Detection of fetal abnormalities with other indicators or risk factors (Low human chorionic gonadotrophin (HCG) and high-unconjugated estriol (uE3) are predictive of an increased risk for Trisomy 18. Echogenic bowel grades 2 and 3 are indicative of an increased risk of cystic fibrosis and Trisomy 21)
- Determination of fetal presentation
- Suspected multiple gestation, serial evaluation of fetal growth in multiple gestation
- Suspected hydatidiform mole
- Suspected fetal death
- Suspected uterine abnormality
- Suspected abrupt placenta
- Follow-up evaluation of placental location for identified placenta previa

For information on diagnostic radiology procedures that require prior authorization, please refer to Chapter 22, Independent Radiology.

Emergency Services For Non-Citizens

Miscarriages

Miscarriages are not currently billable electronically. Requests concerning miscarriages for aliens who are not eligible for pregnancy or full coverage Medicaid continue to be processed manually, until further notice. Aliens, who had miscarriages, must continue to present bills timely (within three months) to the SOBRA worker, who determines eligibility; then forwards information to Central Office for manual processing. Providers will receive a check from

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Medicaid for miscarriages as well as other alien services approved for reimbursement.

Delivery Services Billable Through Gainwell

For CMS-1500 (formerly HCFA-1500) medical claims, the following procedures are covered:

- 59409-vaginal delivery only
- 59612-vaginal only, after previous C-section
- 59514-c-section only
- 59620-c-section only, after attempted vaginal, after previous c-section
- 01960-vaginal anesthesia
- 01961-c-section anesthesia
- 01967-neuraxial labor analgesia/anesthesia
- 62319-epidurals

For UB-04 inpatient claims, the following per diem is covered:

- Up to 2 days for vaginal delivery
- Up to 4 days for C-section delivery.

Allowable diagnosis codes for CMS 1500 or UB-04 are:

For ICD-9

- V270-V279
- V300-V3921
- 65100-65993
- 6571-6573.

For ICD-10

- Z37.3-Z37.4
- Z37.50-Z37.54
- Z37.59
- Z37.60-Z37.64
- Z37.69
- Z37.7
- Z37.9
- Z38.00-Z38.5
- Z38.61-Z38.69
- Z38.7-Z38.8
- O09.40-O09.529
- O30.001-O36.93X9
- O40.1XX0-O43.93
- O61.0-O61.9
- O64.1XX0-O64.9XX9
- O65.0-O66.6
- O68

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- O75.2-O75.3
- O75.5
- O75.89-O75.9
- O76-O77.9

Allowable surgical codes for CMS 1500 or UB-04 are:

For ICD-9

- 740-7499

For ICD-10

- 10D00Z0-10D00Z2
- 10D07Z3-10D07Z8

24.7.7 Care Coordination Services

Pregnant women are eligible for care management services which include case management or care coordination services provided by the ACHN program to help improve maternal health outcomes. Services provided include but are not limited to eligibility assistance, choosing a physician, and appointment scheduling/reminders. Visit www.Medicaid.Alabama.gov, there select the ACHN tab, select ACHN Recipients and, under Program Resources select "ACHN Contact for Recipients and Interactive Map". On the map select the county that the Recipient resides to obtain the contract number for the applicable ACHN.

24.7.8 Assistant Surgeon Fees

The assistant surgeon fees for cesarean (C-section) deliveries are to be billed fee for service.

24.7.9 Anesthesia Services

Anesthesia services include anesthesia services performed by an anesthesiologist or the delivering physician that are not medically contraindicated.

24.7.10 Other Billable Services

Services provided outside the scope of the global fee that may be billed separately are listed below:

Separately Billable Service	Description
Drugs	Family planning or general drugs (for example, oral contraceptives or iron pills) prescribed by a provider with a written prescription to be filled later may be billed on a fee-for-service basis. In addition, women on Plan First have the option of obtaining oral contraceptives, the contraceptive ring, or the contraceptive patch, with a prescription from a private provider, at a Medicaid-enrolled community/outpatient pharmacy. Injections administered by the physician or outpatient facility can be billed on a fee-for-service basis (for example, Rhogam or Iron). Smoking cessation products for pregnant women will be covered after prior authorization through the Pharmacy Administrative Services contractor. Refer to Appendix Q Tobacco Cessation for additional information. The recipient must be enrolled and receiving counseling services through the Alabama Department of Public Health Quitline. Approval will be granted up to 3 months at a time.

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Lab Services	All lab services except hemoglobin, hematocrit, and chemical urinalysis.
Radiology	All radiology services are outside of the global fee unless performed during an inpatient stay or for ultrasounds and non-stress tests. The professional component for radiology services is a component of the primary contractor global fee and should be billed separately to the primary contractor with the exception of teaching hospitals.
Dental	Effective **October 1, 2022**, Alabama Medicaid will reimburse for dental services rendered to pregnant adults who are ages 21 and older during pregnancy, and during postpartum in accordance with 42 CFR § 440.210(a)(3), when the services are rendered by Alabama Medicaid enrolled dental providers. Postpartum for dental services is 60 days post delivery or end of pregnancy. Services will not change for individuals under age 21.
Physician	Physician fees for family planning procedures (for example, sterilization), and genetic counseling. Claims for circumcision, standby and infant resuscitation may be billed under the mother's name and number on a fee-for-service basis.
Family Planning Services	Appropriate Family planning services with family planning procedure code or indicator. Eligible recipients will continue to have the option of receiving family planning services from the Alabama Department of Public Health or a Federally qualified Health Center. All family planning services must meet program guidelines and eligibility criteria.
Home Visit (99347-FP)	The home visit is a brief evaluation by a medical professional in the home of an established recipient and is for the purpose of providing contraceptive counseling (using the PT+3 teaching method) and administration/issuance of contraceptive supplies. The home visit is for postpartum women during the postpartum period and usually occurs within 7-14 days after delivery. A home visit is limited to one per postpartum period **and usually occurs within 7-14 days after delivery. A home visit is not a covered service for recipients with Plan First eligibility and can only be provided as a family planning service by Medicaid eligible family planning providers to eligible recipients.** To qualify for reimbursement for the home visit: • Medical professionals who are licensed to administer medications such as oral contraceptives or to give injections must provide the home visit. • The home visit must include: brief medical histories: family, medical, contraceptive, and OB/GYN, blood pressure and weight check, contraceptive education and counseling using the PT+3 teaching method assuring that the recipient: − understands how to use the method selected, − how to manage side effects/adverse reactions, − when/whom to contact in case of adverse reactions, and the importance of follow-up. − scheduling of a follow-up visit in the clinic if needed − issuance or prescription of contraceptive supplies as appropriate. The recipient must give her signed consent for this visit.

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Extended Family Planning Counseling Visit (99212-FP)	The extended family planning counseling visit is a separate and distinct service consisting of a minimum of 10 face-to-face minutes of extended contraceptive counseling using the PT+3 teaching method. **The extended family planning counseling visit is for postpartum women during the postpartum period and is performed in conjunction with the 6-week postpartum visit in the office/clinic setting. An extended family planning counseling is limited to once during the post-partum period, and is not available for women who have undergone a sterilization procedure or Plan First eligible recipients on the Plan First Program.** The counseling services are those provided **above and beyond the routine contraceptive counseling that is included in the postpartum visit.** The purpose of this additional counseling time is to take full advantage of the window of opportunity that occurs just after delivery when the physical need for pregnancy delay is at a peak**.** **An Extended Family Planning Counseling Visit is not covered for Plan First recipients and can only be provided as a family planning service by eligible family planning providers to eligible recipients.** The following services are required: • Contraceptive counseling and education • STD/HIV risk screening and counseling, and • Issuance of contraceptive supplies. **NOTE:** In the event of a premature delivery or miscarriage, the EDC, "Expected Date of Confinement", must be documented on the claim form in block 19 in order to be reimbursed for procedure code 99212-FP. All visits must be documented in the recipient's chart and reflective of the treatment and care provided.
Emergency Services	Outpatient emergency room services (including the physician component) (claims containing a facility fee charge of 99281, 99282, 99283, 99284, or 99285) and associated physician charges (99281-99288) will be reimbursed separately from the global fee. Access to emergency services will not be restricted by the Maternity Care Program.
Transportation	Transportation as allowed under the Alabama Medicaid State Plan may be billed on a fee-for-service basis.
Mental Health	**S**creening, **B**rief Intervention, and **R**eferral to **T**reatment (SBIRT) are services designed to identify individuals who are at risk for development of substance use disorders, assist individuals in implementing strategies to reduce the potential for development of substance use disorders, and refer individuals who have identified needs for substance abuse treatment to specialized substance abuse treatment providers. Note: The intent of SBIRT is referral for Substance Abuse to include alcohol and drug abuse as smoking cessation is covered in the Maternity Care Program under Care Coordination Services. Screening: A full screen, as reimbursable through this benefit, is a structured process used to identify an individual whose current use of alcohol and/or other drugs creates a clearly defined risk for harm in some life dimension. A non-reimbursable pre-screening process must provide documentation of the need for a full screen. The pre-screening process may consist of, as few as, one to two brief questions incorporated into a general health questionnaire; a valid and reliable short screening tool; observations of attending medical personnel; interview and self-report; laboratory results; and/or concerns expressed by significant others. The full screen must be conducted utilizing an authorized, evidence-based screening tool with established reliability and validity in the identification of individuals who are at risk for developing substance use disorders. The tool must also provide enough information to establish an appropriate level of intervention in relation to each individual's identified risk factors. Authorized tools that may be used to conduct the full screen include the following: • Alcohol, Smoking, and Substance Involvement Test (ASSIST) • Drug Abuse Screening Test (DAST)

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- Alcohol Use Disorders Identification Test (AUDIT)
- Car, Relax, Alone, Forget, Family or Friends, Trouble (CRAFFT) Questionnaire
- Problem Oriented Screening Instrument for Teenagers (POSIT)
- UNCOPE Substance Abuse Screening

Additional tools that conform to the criteria specified above may be utilized to provide the full screen. Prior to use, however, each tool not listed above must be reviewed and authorized for use by the Alabama Medicaid Agency.

The full screening process includes the provider's evaluation of the results and an explanation of these results to the individual who has been screened. The provider must clearly explain the level of risk associated with the identified alcohol and/or drug use pattern, and describe the corresponding implications within the context the individual's health and other life dimensions.

The provider's response to low risk substance use shall be provided during the screening process as according to the identified needs of the individual. This may include, but is not limited to, the dissemination of material that provides information on the risks associated with drinking and drug use, for example:

- Potential alcohol and drug interactions with medications the individual is taking.
- The potential for exacerbation of a health condition with alcohol and drug use.
- The potential impact of alcohol or drug use on pregnancy.

If the individual has a positive full screen, indicative of a moderate to high risk for a substance use disorder, the provider must be prepared to conduct or obtain brief intervention services during this same visit.

BRIEF INTERVENTION

A brief intervention is an organized encounter that includes, at a minimum, a provider and an individual who has been identified through a full screening process as being at moderate to high risk for development of a substance use disorder. Through the use of motivational strategies with demonstrated effectiveness, the goals of a brief intervention are to increase the individual's awareness and insight regarding current alcohol and/or drug use; to establish acceptance of a need for change; and to support the individual in development and implementation of a plan for change.

The brief intervention may consist of a single brief (15 minutes) session or multiple brief sessions dependent upon the unique needs of each individual. Referrals for specialized substance abuse treatment services are provided in conjunction with brief interventions. During any brief intervention, including the first session, the provider must be prepared to make a direct referral to a specialized substance abuse treatment provider for individuals who are at high risk for severe substance use and related consequences. Referrals must be initiated as soon a need for such is established.

SERVICE UNITS/LIMITS

Screening: H0049
Service Unit: Episode
Limit: One per pregnancy

Providers may bill for time that is spent face-to-face administering an authorized screening tool, discussing the screening results, and providing recommendations for further actions. Providers may not bill for the time during which an individual self-administers a screening tool.

Brief Intervention: H0050
Service Unit: 15 minutes
Limit: 1/day, 2/pregnancy

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	Providers may bill for time that is spent face-to-face implementing strategies to assist individuals with moderate to high risks for development of substance use disorders in behavior modification that supports risk reduction. Allowable strategies include efforts made by the provider to assist the individual in accessing specialty substance abuse treatment services when there is an identified need for such. **Restrictions:** SBIRT services are not a covered benefit for: • Smoking and tobacco abuse. • Individuals who have been diagnosed with a substance use disorder. • Individuals who have had previous and/or are now receiving treatment for a substance use disorder. **Service Documentation:** Documentation of services provided shall incorporate the following: • The need for and method of identification of the need for SBIRT as established during a pre-screening process. • Identification of the screening tool used to conduct the full screening process. • The results of the full screening process. • Brief intervention goals unique to each individual. • Summary report of each brief intervention session conducted, including the implementation of established motivational strategies. • Referrals made and outcomes. • Follow-up services provided. **Approved Providers:** Coverage of Screening, Brief Intervention, and Referral for Treatment (SBIRT) for pregnant women is covered in conjunction with antepartum care provided by physicians, physician employed nurse practitioners, nurse midwives, physician-employed physician assistants and FQHCs. Prior to offering the services health care professionals must complete an online tutorial which can be accessed at http://www.mh.alabama.gov. The Mental Health and Substance Abuse Services Division of the Alabama Department of Mental Health will notify the Medicaid Maternity Care Program of health care professionals' successful completion of the tutorial. Procedure codes H0049 (screening for substance use) and H0050 (brief intervention and referral to treatment) will then be billable for the health care professional who has successfully completed the online tutorial. An ICD-10 code of Z331 must be billed by the provider on the claim form.
Screening for Perinatal Mood Disorders:	Effective January 1, 2026, Alabama Medicaid will reimburse for perinatal mood disorder screening services rendered to pregnant recipients over the age of 21, during the prenatal and postpartum periods as follows: • Maternity Care providers are allowed to bill 96127 • Procedure code 96127 is allowed once per date of service and up to 4 submissions per perinatal period • Claim submission requires a pregnancy or postpartum diagnosis **and** diagnosis code Z13.30 • Claim must be billed outside of the maternity global for payment by Medicaid The following screening tools are eligible for reimbursement under this provision: o Patient Health Questionnaire-2 or Patient Health Questionnaire-9 (PHQ-2 or PHQ-9) o Edinburgh Postnatal Depression Scale (EPDS) o Hamilton Rating Scale for Depression (HAM-D) o Montgomery-Asberg Depression Rating Scale (MADRS) o Beck's Depression Inventory (BDI) o Generalized Anxiety Disorder 7 (GAD-7) o Mood Disorder Questionnaire (MDQ) o Primary Care PTSD Screen for DSM-5 (PC-PTSD-5) o Duke Anxiety-Depression Scale (DUKE-AD)
Referral to Specialists	MCPs may provide referrals to specialists. Services provided by non-OB specialty physicians (i.e. cardiologists, endocrinologists) for problems complicated or exacerbated by pregnancy can be billed fee-for-service by the provider of service. A general/family practitioner is not considered a specialty provider. A Board Certified Perinatologist is considered a specialty provider and may bill fee-for-

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	service for high risk patients only. Refer to the Chapter 28, Physicians Chapter, for billing information.
Non-Pregnancy Related Care	Services provided that are not pregnancy-related may be billed fee-for-service. A recipient's age, health care requirements, and place of residence may further define his or her eligibility for Medicaid covered services. For this reason, it is very important that the providers verify recipient eligibility and ensure they understand all aspects of the eligibility response. A provider may reference the fee schedules for a list of covered services on the following link: http://medicaid.alabama.gov/content/Gated/7.3G_Fee_Schedules.aspx.The fee schedules are not an all-inclusive list of procedure codes covered by the Agency. Reference Chapter 4, Obtaining Prior Authorization (PA) for PA requirements.
Tobacco Cessation Face-To-Face Counseling	The Alabama Medicaid Agency covers smoking cessation benefits for Medicaid-eligible pregnant women. Medicaid will reimburse for up to four face- to-face counseling sessions in a 12-month period. The reimbursement period will begin in the prenatal period and continue through the postpartum period. Documentation must support each counseling session. 28 Face-to-face counseling services must be provided: • By or under the supervision of a physician; • By other health care professionals who are legally authorized to furnish such services under State law and within their scope of practice and who is authorized to provide Medicaid coverable services other than tobacco cessation services. Refer to Appendix Q Tobacco Cessation for additional information.
Long Acting Reversible Contraception (LARC)	Effective for dates of service June 4, 2019, and thereafter Alabama Medicaid will reimburse the cost of the long acting reversible contraceptive to the facility when provided in the inpatient hospital setting immediately after a delivery or up to the time of the inpatient discharge for postpartum women, or in an outpatient setting immediately after discharge from the inpatient hospital for postpartum women. The insertion of the device/drug implant will be billable to Medicaid by both the physician and hospital for reimbursement. Refer to Chapter 19 Hospital for additional information. For questions regarding hospital billing contact the Medical Services Division. For questions regarding physician billing contact Jean Wackerle, Associate Director, Physicians Program, at (334) 242-2312 or via email Jean.Wackerle@medicaid.alabama.gov
Chromosomal Studies	Medicaid can pay for these studies on eligible prospective mothers in an effort to identify conditions that could result in the birth of an abnormal child. Chromosomal studies (amniocentesis) on unborn children being considered for adoption are not covered.
Obstetrical Ultrasounds	Effective 10/1/2019, Medicaid no longer requires prior authorization for obstetrical ultrasounds. All ultrasounds must be medically necessary with medical diagnosis documented supporting the benefit of the ultrasound procedure.

24.8 Prior Authorization and Referral Requirements

MCPs may provide referrals to specialists. Referrals to specialty providers for a pregnant recipient (i.e., Cardiology, Endocrinology, etc.) are paid fee-for-service.

Reference Chapter 4, Obtaining Prior Authorization (PA) for PA requirements.

24.8.1 MCP SELECTION REFERRAL REQUIREMENTS

All maternity claims must have a MCP selection referral number from the ACHN to receive payment from Medicaid. Although MCPs will already have the ACHN NPI number, it is the responsibility of the MCP to ensure a referral is in medical record and contact has been made with the ACHN. This

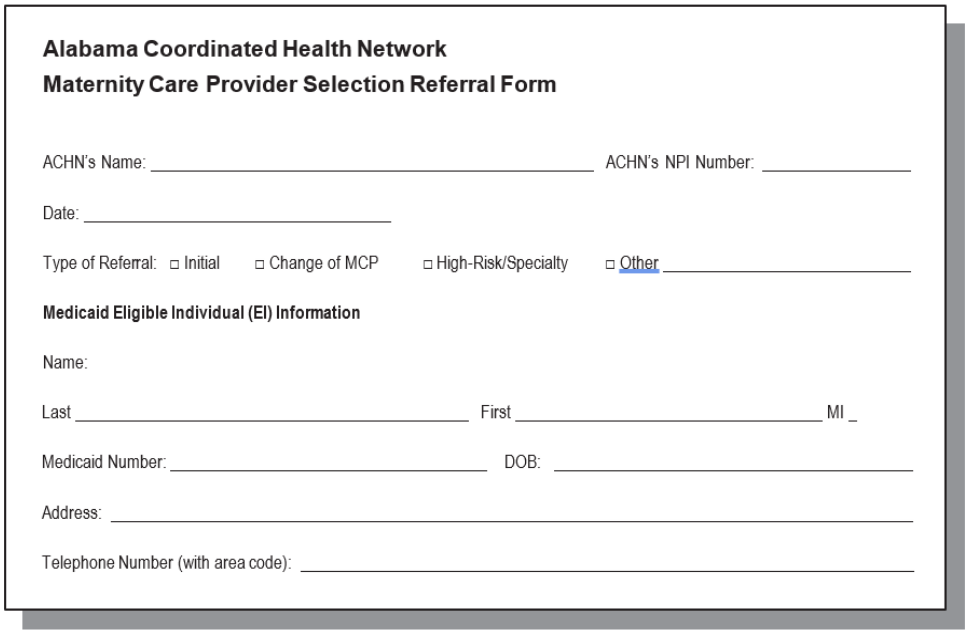
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ensures collaborative communication between the MCP and the ACHN for quality health outcomes.

Exception: Medicaid Recipients that are not assigned to an ACHN on the date of service will not require a MCP referral from the ACHN for reimbursement.

NOTE:
In emergency circumstances, maternity claims submitted with an emergency indicator (Certified Emergency – Service Authorization Exception Code – 3) will not require a MCP referral from an ACHN for reimbursement.

Sample ACHN MCP Referral Form



Alabama Coordinated Health Network
Maternity Care Provider Selection Referral Form

ACHN's Name: _____ ACHN's NPI Number: _____

Date: _____

Type of Referral: □ Initial □ Change of MCP □ High-Risk/Specialty □ Other _____

Medicaid Eligible Individual (EI) Information

Name:

Last _____ First _____ MI _

Medicaid Number: _____ DOB: _____

Address: _____

Telephone Number (with area code): _____

24.9 Spontaneous Abortions

Spontaneous abortions do not require a MCP referral number from the ACHN. However, the MCP should notify the ACHN when a pregnancy ends due to a spontaneous abortion.

24.10 Cost Sharing (Copayment)

Copayment does not apply to pregnancy-related services provided for pregnant women, but a relevant pregnancy or postpartum diagnosis code must be on the claim.

24.11 Completing the Claim Form

To enhance the effectiveness and efficiency of Medicaid processing, providers should bill Medicaid claims electronically.

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Refer to Appendix B, Electronic Media Claims Guidelines, for more information about electronic filing.

NOTE:

When filing a claim on paper, a CMS-1500 claim form is required.

This section describes program-specific claims information. Refer to Chapter 5, Filing Claims, for general claims filing information and instructions.

24.11.1 Time Limit for Filing Claims

Medicaid requires all claims from MCPs to be filed within one year of the date of service. Refer to Chapter 5, Filing Claims, for more information regarding timely filing limits and exceptions.

24.11.2 MCP Selection Referral Number

All maternity claims must have a MCP selection referral number from the ACHN to receive payment from Medicaid. When filing an electronic claim using Medicaid's web portal, the MCP Selection Referral Number should be entered into the "referring physician" field. If using a vendor/clearinghouse software, the provider will need to confirm with the vendor on where to enter the MCP selection referral number.

NOTE:

If you use a billing software to submit claims and are not currently set up to insert the referring provider information, you may contact the EMC Helpdesk at 1-800-456-1242 for assistance in getting the software setup properly.

For paper claim submissions, the MCP selection referral number should be entered as follows:

- **CMS-1500 Claims-** enter the name of the referring ACHN i block 17. Enter the secondary ID (Medicaid ID) in block 17A. The secondary ID may not always be necessary. Enter the ACHN's Referral Number (NPI) in block 17B.

- **UB-04 Claims-** Enter the ACHN's Referral Number (NPI) in block 78.

24.11.3 Diagnosis Codes

A pregnancy or postpartum diagnosis code, primary or secondary, must be used when billing maternity care services. At least two pregnancy diagnosis must be included on the claim. One of the diagnosis must be the weeks of gestation.

The *International Classification of Diseases - 10th Revision - Clinical Modification* (ICD-10-CM) manual lists required diagnosis codes. These

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manuals may be obtained by contacting the American Medical Association, AMA Plaza 330 North Wabash Ave, Suite 39300 Chicago, IL 60611-5885, or 1-800-621-8335.

NOTE:

ICD-10 diagnosis codes must be listed to the highest number of digits possible (3, 4, or 5 digits). Do not use decimal points in the diagnosis code field.

24.11.4 Procedure Codes, Modifiers, and Rates

MCPs are to bill all claims to Gainwell utilizing the appropriate CPT code. The following MCP reimbursement table conveys ACHN delivery rates (effective 10/1/2019) for rural and urban delivery procedure codes:

Code	*Modifier*	*Description*	*ACHN Rural*	*ACHN Urban*
59400	U9	Routine obstetric care including antepartum care, vaginal delivery **(delivery at 39 weeks of gestation or later)** (with or without episiotomy or forceps) and postpartum care	$2,090	$1,690
59400	UD	Routine obstetric care including antepartum care, vaginal delivery **(medically necessary delivery prior to 39 weeks of gestation)** (with or without episiotomy or forceps) and postpartum care	$2,090	$1,690
59400	UC	Routine obstetric care including antepartum care, vaginal delivery **(non-medically necessary prior to 39 weeks of gestation)** (with or without episiotomy or forceps) and postpartum care	$2,090	$1,690
59409	U9	Vaginal delivery only **(delivery at 39 weeks of gestation or later)** (with or without episiotomy and/or forceps)	$1,640	$1,340
59409	UD	Vaginal delivery only (**medically necessary delivery prior to 39 weeks of gestation**) (with or without episiotomy and/or forceps)	$1,640	$1,340
59409	UC	Vaginal delivery only (**non-medically necessary delivery prior to 39 weeks of gestation**) (with or without episiotomy and/or forceps)	$1,640	$1,340
59410	U9	Vaginal delivery **(delivery at 39 weeks of gestation or later)** and postpartum care only	$1,690	$1,390
59410	UD	Vaginal delivery **(medically necessary delivery prior to 39 weeks of gestation)** and postpartum care only	$1,690	$1,390
59410	UC	Vaginal delivery **(non-medically necessary prior to 39 weeks of gestation)** and postpartum care only	$1,690	$1,390

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Code	Modifier	Description	ACHN Rural	ACHN Urban
59510	U9	Routine obstetric care including antepartum care, cesarean delivery **(delivery at 39 weeks of gestation or later)** and postpartum care	$2,090	$1,690
59510	UD	Routine obstetric care including antepartum care, cesarean delivery **(medically necessary delivery prior to 39 weeks of gestation)** and postpartum care	$2,090	$1,690
59510	UC	Routine obstetric care including antepartum care, cesarean delivery **(non-medically necessary prior to 39 weeks of gestation)** and postpartum care	$2,090	$1,690
59514	U9	Cesarean delivery only **(medically necessary delivery prior to 39 weeks of gestation)**	$1,640	$1,340
59514	UD	Cesarean delivery only (**medically necessary delivery prior to 39 weeks of gestation**)	$1,640	$1,340
59514	UC	Cesarean delivery only **(non-medically necessary delivery prior to 39 weeks of gestation)**	$1,640	$1,340
59515	U9	Cesarean delivery **(delivery at 39 weeks of gestation or later)** and postpartum care only	$1,690	$1,390
59515	UD	Cesarean delivery **(medically necessary delivery prior to 39 weeks of gestation)** and postpartum care only	$1,690	$1,390
59515	UC	Cesarean delivery **(non-medically necessary delivery prior to 39 weeks of gestation)** and postpartum care only	$1,690	$1,390
59610	U9	Routine obstetric care including antepartum care, vaginal delivery **(delivery at 39 weeks of gestation or later)** (with or without episiotomy and/or forceps) and postpartum care, after previous cesarean delivery	$2,090	$1,690
59610	UD	Routine obstetric care including antepartum care, vaginal delivery (**medically necessary delivery prior to 39 weeks of gestation**) (with or without episiotomy and/or forceps) and postpartum care, after previous cesarean delivery	$2,090	$1,690
59610	UC	Routine obstetric care including antepartum care, vaginal delivery (**non-medically necessary delivery prior to 39 weeks of gestation**) (with or without episiotomy and/or forceps) and postpartum care, after previous cesarean delivery	$2,090	$1,690

Medicaid will reimburse maternity services to FQHCs and RHCs via PPS rates and Fee-For-Service rates. For more reimbursement information, refer to the FQHC and RHC chapters of the Provider Billing Manual.

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NOTE:
Nurse Midwives will be reimbursed 80% of the physician rate.

Rural rates apply to the county location of the MCP's offices. A map showing urban and rural counties for providers may be accessed on the Medicaid website at www.medicaid.alabama.gov, then select the ACHN tab, ACHN Providers, Medicaid's Designated Urban and Rural Map.

MCP Initial Prenatal and Postpartum Bonus Payments:

Medicaid will pay bonus payments for the following procedure codes on a separate claim:

- **H1000**-Initial Prenatal Visit. Actively participating MCPs may bill procedure code **H1000** if the following criteria is met:
  - the Medicaid recipient has a confirmed pregnancy by a medical professional and/or lab test on or before the date of service (confirmatory pregnancy testing is not required for Medicaid eligibility) and
  - the Medicaid recipient completes a gynecology/obstetrics medical visit within 90 days (12 weeks) of the last menstrual period.
  - $150 will be paid once per pregnancy.

- **G9357-**Postpartum Visit. Actively participating Maternity Care Providers (MCPs) may bill procedure code **G9357 (plus applicable modifier)** if the following criteria is met:
  - The Medicaid recipient has delivered (including miscarriages/spontaneous abortions).
  - A paid delivery claim must be processed by the Medicaid Agency.
  - The Medicaid recipient completes a visit with the MCP between 7 and 84 days after the delivery (including miscarriages/spontaneous abortions).
  - $125 will be paid if the Medicaid recipient completes a postpartum visit between 7 and 21 days after delivery (including miscarriages/spontaneous abortions). **You must use modifier U1 to indicate the first postpartum visit between 7-21 days.**
  - An additional $125 will be paid if the Medicaid recipient completes a postpartum visit between 22 and 84 days after delivery (including miscarriages/spontaneous abortions). **You must use modifier U2 to indicate the second postpartum visit between 22-84 days**.
  - The first postpartum visit is not required to be eligible for the second postpartum visit.
  - There must be at least 14 days between the first and second postpartum visits.
  - Services rendered to non-citizens are not eligible for the Postpartum (G9357) bonus payments. Claims billed for services rendered to non-citizens using the mentioned HCPCS codes will be denied.
  - A maximum of two (2) postpartum bonus payments will be paid per Medicaid recipient per pregnancy. Duplicate postpartum

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bonus payments will not be paid for the same recipient during the same pregnancy.

- If a provider files procedure code G9357 between 7 and 84 days of delivery and receives a denial, the provider may submit an override request. The override request must be sent to:

Alabama Medicaid Agency
Managed Care Division
Attention: Maternity Program
501 Dexter Avenue
P.O. Box 5624
Montgomery, AL 36103-5624

Deleted: ~~Linda White~~
Added: Maternity Program

Nurse Practitioners, Physician Assistants, and Nurse Midwives will receive 80% of the physician rate for these Bonus Payments. These Bonus Payments also apply to FQHCs and RHCs. If the Medicaid recipient is not assigned to an ACHN on the date of service, the MCP will not be eligible for the postpartum bonus payments.

NOTE:

Non-citizens are not eligible for the Initial Prenatal (H1000) nor the Postpartum (G9357) bonus payments. Claims billed for non-citizens using the mentioned procedure codes will be denied.

Claims:

The (837) Professional and Institutional electronic claims and the paper claim have been modified to accept up to four Procedure Code Modifiers. Claims for maternity care services are limited to the following procedure codes and modifiers:

NOTE:

Claims that are submitted for obstetric delivery procedure codes **59400, 59409, 59410, 59510, 59514, 59515, 59610, 59612, 59614, 59618, 59620, or 59622** will require one of the following modifiers:

U9-Delivery at 39 weeks of gestation or later

UD-Medically necessary delivery prior to 39 weeks of gestation

UC-Non-medically necessary delivery prior to 39 weeks of gestation

Claims for deliveries that are submitted without one of the required modifiers will be denied.

Appropriate Use of Modifiers

Please refer to this CMS link for more information regarding NCCI edits: https://www.medicaid.gov/medicaid/program-integrity/ncci/edit-files/index.html

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Modifier 25 (Significant, Separately Identifiable Evaluation and Management Service by the Same Physician on the Same Day of the Procedure or Other Service)

It may be necessary to indicate that on the day a procedure or service identified by CPT code was performed, the patient's condition required a significant, separately identifiable E&M service above and beyond the other service provided or beyond the usual preoperative and postoperative care associated with the procedure that was performed. A significant, separately identifiable E&M service is defined or substantiated by documentation that satisfies the relevant criteria for the respective E&M service to be reported.

Reimbursement for Services

Global/delivery-only fees paid by Medicaid to the MCP represent payment in full. **Recipients may not be billed for any services covered under this program.**

NOTE:
Providers are to bill the delivery only procedure codes for pregnant non-citizens.

High Risk Transfers/Reimbursement Methodology

Routine maternity care services provided to a recipient by a MCP before and after the transfer of a recipient to a teaching physician as defined in Section 4,19-B of the State Plan or to a Medicaid enrolled Board Certified Perinatologist will be reimbursed **fee-for-service.** Perinatologists will not require a MCP referral when billing for ultrasounds appending the 26 modifier.

24.11.5 Place of Service Codes

The following place of service code applies when filing claims for maternity care services:

POS Code	*Description*
11	Office
21	Inpatient Hospital

24.12 Reporting Parity and Gravidity for MCPs

The Medicaid Agency uses the American College of Obstetricians and Gynecologists (ACOG) definition for parity and gravidity. According to ACOG, gravidity is defined as the number of pregnancies, current and past, regardless of the pregnancy outcome. ACOG's definition of parity is the number of pregnancies reaching 20 weeks and 0 days of gestation or beyond, regardless of the number of fetuses or outcomes. In cases of multiple pregnancies, parity is only increased with birth of the last fetus.

Hence, parity cannot be greater than gravida. MCPs should adhere to the ACOG's current definitions to report consistent information. For additional information on ACOG's definitions, visit the following link: https://www.acog.org/practice-management/health-it-and-clinical-informatics/revitalize-obstetrics-data-definitions

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24.13 For More Information

This section contains a cross-reference to other relevant sections in the manual.

Resource	Where to Find It
Becoming a Medicaid Provider	Chapter 2
Verifying Recipient Eligibility	Chapter 3
Obtaining Prior Authorization	Chapter 4
CMS-1500 Claim Filing Instructions	Chapter 5
Medical Necessity/Medically Necessary Care	Chapter 7
Physicians	Chapter 28
Long Acting Reversible Contraception (LARC)	Chapter 19
Electronic Media Claims (EMC) Submission Guidelines	Appendix B
AVRS Quick Reference Guide	Appendix L
Alabama Medicaid Contact Information	Appendix N
Tobacco Cessation	Appendix Q

24.14 Contact Information Summary

For general ACHN billing questions or to request an application package, call the Provider Assistance Center: 1 (800) 688-7989.

To disenroll from the program, the request may be faxed to Gainwell Provider Enrollment: (334) 215-4298 or mailed to Gainwell Provider Enrollment, PO Box 241685, Montgomery, AL 36124

To obtain recipient information on eligibility, benefit limits, or coverage, call the Provider Assistance Center: 1 (800) 688-7989

Automated Voice Response System: 1 (800) 727-7848

To address program and policy questions, for recipient language interpretation services or to report patients enrolled in ACHN who should not be enrolled, call the Recipient Call Center: 1(800) 362-1504

ACHN forms may be requested on Medicaid's website at www.medicaid.alabama.gov.

For written correspondence to the Agency: Alabama Medicaid Agency, 501 Dexter Avenue, P.O. Box 5624, Montgomery, AL 36103-5624.

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